

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: The patient is at 32 weeks gestation and has a medium risk for both preeclampsia and macrosomia. Key findings include an elevated systolic blood pressure (142 mmHg) and proteinuria (dipstick 1), which require further evaluation. Fasting glucose is slightly elevated, and pre-pregnancy BMI is high, contributing to the macrosomia risk.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	142/79 mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	1	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=0, Vision=0, RUQ Pain=0, Swelling=0	Severe features increase escalation priority.
Glucose	Fasting=99 mg/dL 1hr PP=126 mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	5.6	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=38 BMI=42.4 GDM=0	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=29 min Sleep=6.1 hrs	Lifestyle factors influence glycemic response.
Food Log	paneer salad	Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: Patient ID: TEST_MED_MED, Gestational Age: 32 weeks. Preeclampsia Risk Tier: Medium. Macrosomia Risk Tier: Medium. Key vitals: Systolic BP 142 mmHg, Diastolic BP 79 mmHg, Proteinuria dipstick 1. Fasting Glucose 99 mg/dL. No severe headache, vision changes, upper abdominal pain, or swelling of face/hands reported. Elevated systolic BP and presence of proteinuria warrant further clinical evaluation for preeclampsia per ACOG guidelines. Elevated pre-pregnancy BMI (42.4 kg/m²) and fasting glucose suggest continued monitoring and lifestyle guidance for macrosomia prevention.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

